FIRST VISIT MEDICAL SERVICES PA
390 NE 191st St STE 8349
Miami, Florida - 33179-3899
Tel: 754-315-7082, Fax: 754-315-7082

Patient	: **Lewanda Brown**	Record Id :	**PAT6499**
Date of Birth	: **Aug 26, 1966**	Age / Gender :	**58** / **F**
Provider	**: Lauren Shaw, FNP-C**	Date of Visit	**: Oct 15, 2024**
Visit Type	**: New Patient Visit**		

Chief Complaints

Neck pain, Low back pain

Related To

Auto Accident - Date: Sep 22, 2024; Place: Georgia; Qualifier: (454) Initial Treatment

History of Present Illness

This is a 58 year old female patient, who presents complaining of neck pain and low back pain following a motor vehicle accident, which occurred on 09/22/2024. The patient was a restrained front passenger and the impact occurred on the rear end passenger side of the vehicle. There was no damage to the seat and the airbags did not deploy. She was able to exit the vehicle independently, and reports there was no loss of consciousness.

Neck pain is a 3-4 on a pain scale of 0-10. Pain is a constant, "pull", sharp type pain. Aggravating factors include various movements of neck. Relieving factors include taking Ibuprofen, chiropractic treatment. Pain radiates down into left shoulder. Denies numbness or tingling in bilateral upper extremities.

Low back pain is a 3 on a pain scale of 0-10. Pain is an intermittent, sharp type pain. Aggravating factors include turning to the left side, turning too far to the right side. Relieving factors include taking Ibuprofen. Pain does radiate down into hips, buttocks, and lower legs. Denies numbness or tingling in bilateral lower extremities. Denies loss of bowel or bladder function.

She was evaluated at the Urgent Care after the accident. The patient is currently receiving chiropractic and neurology treatment at this time. MRI has been ordered by outside provider per patient report.

Past Medical History

PSH:
- Appendectomy
- Lumpectomy

PMH:
- Sciatica

Family and Social History

Family History

Non-contributory.

Social History

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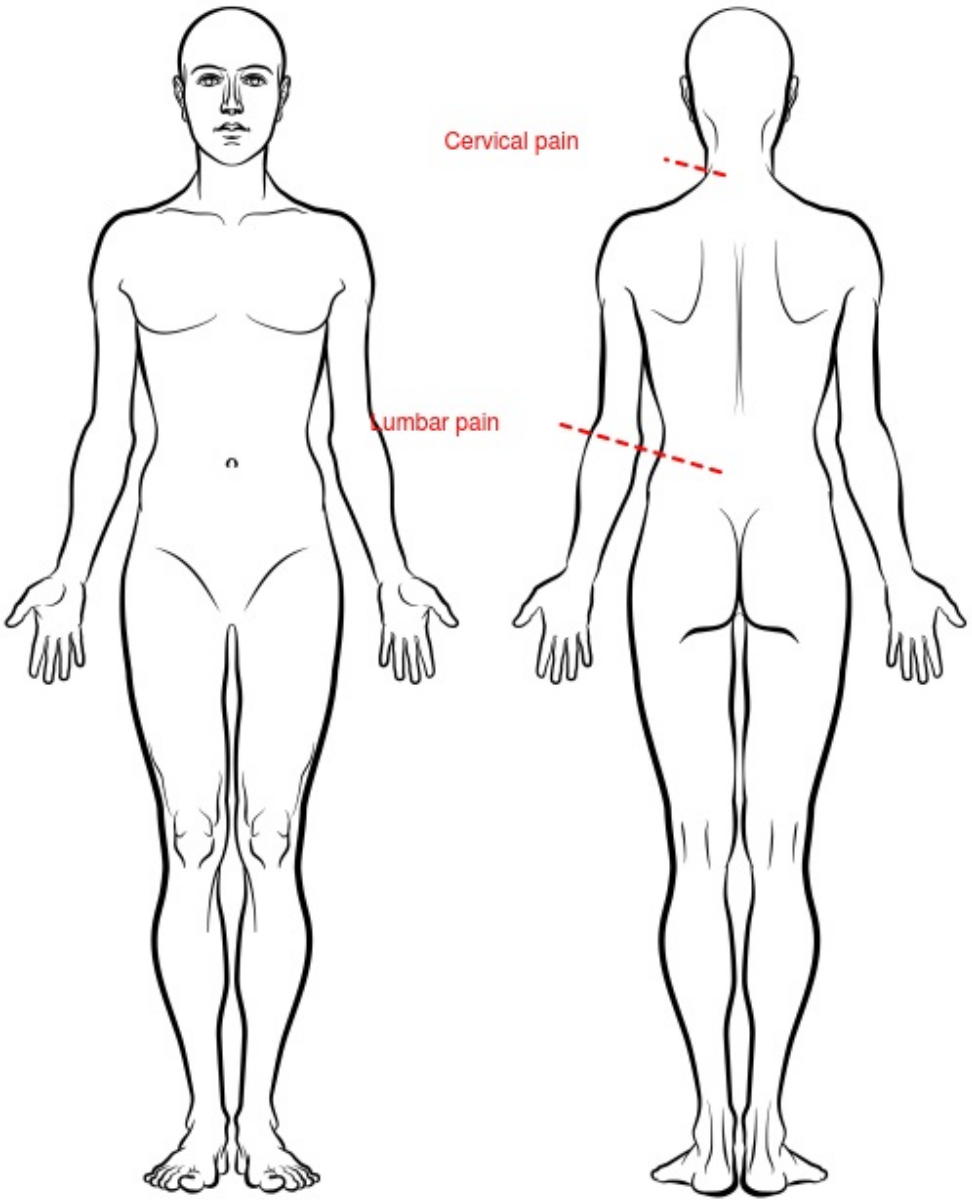
Patient is employed.

Review of Systems

12 -point ROS conducted and all are negative unless otherwise specified in HPI.



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Physical Examination

Vital Signs

Deferred due to telemedicine visit.

Physical exam completed with limitations due to nature of telemedicine visit.

General

Deferred due to telemedicine phone visit.

Pain noted with cervical spine flexion? Yes
Pain noted with extension of cervical spine? Yes
Pain noted with left lateral rotation of cervical spine? Yes
Pain noted with right lateral rotation of cervical spine? Yes
Pain noted with right lateral flexion of cervical spine? Yes
Pain noted with left lateral flexion of cervical spine? Yes
Pain noted with lumbar flexion? Yes
Pain noted with lumbar extension? No
Pain noted with left lateral flexion? Yes
Pain noted with right lateral flexion? No

Neurological

Patient is alert and oriented x4, mood appropriate.

Assessment Notes

The Stanford Concussion Questionnaire was used to assess risk factors consistent with a traumatic brain injury.

Calculated score: 7

Note: Scores above zero are considered abnormal.

A trauma questionnaire was administered and positive for signs/symptoms of PTSD.

Diagnoses

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Patient : **Lewanda Brown**
Date of Birth : **Aug 26, 1966**
Provider **: Lauren Shaw, FNP-C**
Visit Type **: New Patient Visit**

Record Id : **PAT6499**
Age / Gender : **58** / **F**
Date of Visit **: Oct 15, 2024**

Diagnosis	Code	Comments
Cervicalgia	M54.2	
Low back pain, unspecified	M54.50	
Anxiety disorder, unspecified	F41.9	

Instructions

Please visit your nearest Emergency Department for any new or worsening symptoms, including, but not limited to: headaches that worsen, seizures, focal neurological signs, increased drowsiness, difficulty awakening, repeated vomiting, slurred speech, difficulty recognizing people or places, increased confusion or irritability, increased weakness or numbness in arms and/or legs, unusual change in behavior or change in level of consciousness, new or worsening shortness of breath, new or worsening chest pain, or any general feeling of being unwell.

Recommend TBI evaluation due to positive concussion questionnaire score.

Recommend referral to pain management for evaluation and treatment of cervical pain and lumbar pain.

Recommend referral for psychiatric consult due to positive trauma questionnaire.

Recommend conservative therapy, exercise, and stretching activities throughout recovery as tolerated.

Recommend continued follow up with chiropractor and neurology as recommended by their offices.

Attestation
These services were provided under physician supervision.

Referrals

Referral To	Reason
Specialty Referral	MVA

Electronically Signed By Lauren Shaw, FNP-C on Oct 15, 2024 10:45 AM US/Eastern

ADVANCED IMAGING CENTERS

PATIENT:	BROWN, LEWANDA
DOB:	8/26/1966
FILE #:	OPI3407886
PHYSICIAN:	HEINIS, RICARDO
EXAMS:	MRI SPINE LUMBAR W/O CONTRAST
DATE:	10/26/2024 12:30:00 PM

MRI OF THE LUMBAR SPINE:

HISTORY: Low back pain following a motor vehicle collision dated 9/22/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The conus medullaris appears normal. The lordotic curvature of the lumbar spine is preserved. No evidence for abnormal solid or cystic lesions is identified. No prevertebral or paravertebral masses or fluid collections are seen and there is no evidence for abnormal marrow replacing lesion.

There are Schmorl's nodes in the inferior endplates of L4 and L5.

Segmental analysis of the lumbar spine is as follows:

At L1-2, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L2-3, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At L3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At L4-5, there is disc bulging. There are facet osteophytes bilaterally. This results in anterior impression on the thecal sac. There is mild right and moderate left neural foraminal stenosis.

At L5-S1, there is disc bulging. There are mild vertebral osteophytes and facet osteophytes present. This results in anterior impression on the thecal sac. There is moderate right and severe left neural foraminal stenosis.

IMPRESSION:

1. At L3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac.
2. At L4-5, there is disc bulging. There are facet osteophytes bilaterally. This results in anterior impression on the thecal sac. There is mild right and moderate left neural foraminal stenosis.
3. At L5-S1, there is disc bulging. There are mild vertebral osteophytes and facet osteophytes present. This results in anterior impression on the thecal sac. There is moderate right and severe left neural foraminal stenosis.

The definitions in this report, including definitions of disc bulge, disc herniation, protrusion, and extrusion, are from the following peer reviewed journal: Lumbar Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525 2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/JN

ADVANCED IMAGING CENTERS

PATIENT:	BROWN, LEWANDA
DOB:	8/26/1966
FILE #:	OPI3406849
PHYSICIAN:	RISH, BRITTNEY
EXAMS:	MRI BRAIN W/O CONTRAST
DATE:	10/16/2024 2:30:00 PM

MRI OF THE BRAIN WITHOUT CONTRAST:

HISTORY: Headache. Motor vehicle collision 9/22/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained of the brain without contrast.

FINDINGS:
SUPRATENTORIAL STRUCTURES: There are bifrontal subcortical foci of T2 hyperintensity seen within both cerebral hemispheres, particularly within the frontal lobes. A few additional foci are seen as well. There is no evidence for a mass, fluid collection or hemorrhage. The basal ganglia are within normal limits.

POSTERIOR FOSSA: The brainstem is normal in signal intensity. The cerebellum appears within normal limits. The cerebellar folia and sulci are unremarkable. The internal auditory canals appear within normal limits. The seventh and eighth cranial nerves are normal and there is no evidence for cerebellopontine angle mass.

VENTRICULAR SYSTEM: The ventricles are normal in size and shape. There is no evidence for hydrocephalus and there is no evidence for transependymal flow of CSF.

SKULL BASE AND OSSEOUS STRUCTURES: The orbits, paranasal sinuses and temporal bones are within normal limits. There is no evidence for abnormal mass or fluid collection associated with these structures.

VASCULAR STRUCTURES: There is normal signal void within the major vessels of the circle of Willis. The superior sagittal sinus appears unremarkable on this examination.

PITUITARY AND SELLA: There is no evidence for mass.

DIFFUSION WEIGHTED IMAGING: There are no signal abnormalities to suggest an acute ischemic process. There is no evidence for acute small vessel ischemia.

IMPRESSION:

1. There are bifrontal subcortical foci of T2 hyperintensity seen within both cerebral hemispheres, particularly within the frontal lobes. **See Figure 1, Image 19, Series 601.** The arrows are pointing to the bifrontal subcortical foci of T2 hyperintensity. A few additional foci are seen as well. Given the patient's history, the findings are suspicious for post-traumatic brain injury. Differential considerations include stroke/small vessel ischemic change and less likely a demyelinating condition such as multiple sclerosis. Correlate clinically to confirm a diagnosis of traumatic brain injury in this patient.

FIGURE 1

Thank you for your kind referral.

SHAHIN J. KORANGY, M.D.
ELITERAD RADIOLOGY GROUP
FELLOWSHIP TRAINED, NEURORADIOLOGY
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SJK/TR

ADVANCED IMAGING CENTERS

PATIENT:	BROWN, LEWANDA
DOB:	8/26/1966
FILE #:	OPI3407885
PHYSICIAN:	HEINIS, RICARDO
EXAMS:	MRI SPINE CERVICAL W/O CONTRAST
DATE:	10/26/2024 11:45:00 AM

MRI OF THE CERVICAL SPINE:

HISTORY: Neck pain following a motor vehicle collision dated 9/22/2024.

TECHNIQUE: Multisequence T1 and T2 weighted images were obtained.

FINDINGS: The posterior fossa structures are normal. The cervical cord structures are normal. There is loss of the normal lordotic curvature of the cervical spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended. No prevertebral or paravertebral masses or fluid collections are identified.

Segmental analysis of the cervical spine is as follows:

At C2-3, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C4-5, there is disc bulging. There are mild anterior vertebral osteophytes present. This results in anterior impression on the thecal sac. There is no right or left neural foraminal stenosis.

At C5-6, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

At C6-7, there is bulging of the disc. This results in an anterior impression on the thecal sac. There is no central canal stenosis or foraminal stenosis.

At C7-T1, there is no evidence for disc herniation, canal stenosis or neural foraminal stenosis.

IMPRESSION:
1. There is loss of the normal lordotic curvature of the cervical spine. In the correct clinical setting, this may reflect injury. Clinical correlation is recommended.
2. At C3-4, there is bulging of the disc. This results in an anterior impression on the thecal sac.
3. At C4-5, there is disc bulging. There are mild anterior vertebral osteophytes present. This results in anterior impression on the thecal sac.
4. At C6-7, there is bulging of the disc. This results in an anterior impression on the thecal sac.

The definitions in this report, including definitions of disc bulge, disc herniation, protrusion, and extrusion, are from the following peer reviewed journal: Lumbar Disc Nomenclature V2.0, Recommendations of the Combined Task Forces of the North American Spine Society, the American Society of Spine Radiology and the American Society of Neuroradiology, The Spine Journal 14 (2014) 2525 2545. References to causation and permanency follow guidelines established by the American Medical Association. Note that a normal MRI does not exclude certain pathologies, including pathologies involving the nerves and facet joints. A normal MRI should not supersede abnormalities detected with physical exam. Disc herniations are contained herniated discs unless specifically identified as uncontained.

Thank you for your kind referral.

SEAN M. MAHAN, M.D.
ELITERAD RADIOLOGY GROUP
CERTIFIED, AMERICAN BOARD OF RADIOLOGY

SMM/JN

3515 Braselton Hwy E2, Dacula GA 30019 - 5926

Tel: 7708004357, Fax:

PROGRESS NOTE

Patient First Name:	Patient Last Name:	Date of Birth:	Sex:
Lewanda	Brown	08-26-1966	Female
Attending Provider:	**Referring Provider:**	**Visit Date:**	**Chart No.:**
Brittney Rish, NP		10-22-2024	SCL00371
Appointment Location:	**Appointment Location Address:**		
Neurospine Atlanta	3515 Braselton Hwy E2, Dacula GA 30019 - 5926		

Chief Complaint: Follow Up
History of Present Illness
Follow Up
Patient states her headaches have been subsiding, she states her last headache was 1 week ago. She states her headaches are under control and manageable at this time. She no longer take medication for headaches.

She denies: sleep disturbance, dizziness, ear ringing.

Past Medical History
No Known Past Medical History

Surgical History

Lymph node removal may 2024

Family History
Non-contributory Family History.

Current Medication
IBU
methocarbamol

Allergy
No Known Drug Allergies.

Social History
Family: She is married.

Review of Systems
Constitutional Symptoms: Patients appears normal. Denies fever and fatigue.
HEENT: Denies headache, dizziness, blurry vision, light sensitivity, pain in eyes, Ear pain, sensitivity to noise

and ringing in the ears.
Respiratory: Denies trouble breathing.
Cardiovascular: Denies chest pain.
Neurological: Denies seizures, trouble with memory and trouble concentrating. Gait abnormality is not observed.

Vitals
Pulse: 82 per min. **BP Systolic:** 123 mmHg. **BP Diastolic:** 84 mmHg.

Physical Examination

General: The patient appears pleasant, in no apparent distress, well nourished and with good attention to hygiene and body habitus.
Neuro: Oriented to person, place and time.
Head: normocephalic, atraumatic
Respiratory: breathing comfortably on room air without evidence of labored breathing, cyanosis, and/or audible wheezing
Cardiovascular: well perfused without evidence of cyanosis

CN:
II : Visual fields are intact on confrontation.
II-IV-VI: Extraocular movements are full without any nystagmus. Pupils are 3.5 mm in size, round, equal and reactive to light.
V: Facial sensations are intact bilaterally and symmetrically.
VII : Facial symmetry is intact bilaterally.
VIII: Hearing is intact bilaterally.
IX -X: Palate is symmetrical bilaterally.
XI: Shoulder shrug is equal and symmetrical bilaterally.
XII : Tongue is in midline.

Coordination :
Finger to nose, rapid alternating movements are intact.
Gait : Romberg is negative, patient is able to do tandem walking without any difficulties.
Motor system examination:
Tone is normal, strength is 5 over 5 in all the muscle groups in upper and lower extremities. DTRs : +2 equal and symmetrical in all muscle groups.
Plantars: These are flexor bilaterally.

Sensory system examination:
Touch, temperature, pinprick sensations are intact equally and symmetrically in upper and lower extremities.

No occipital nerve tenderness

Assessment and Plan
ICD: Follow up (Z09)
Pt presents today for a FU regarding headaches. As of today pt states that headaches have continued to improve and have subsided. Pt states that her last headache was 1 week ago. She no longer takes any medication for her headaches. She denies: sleep disturbance, dizziness, and ear ringing.
She states her headaches are under control and manageable at this time.
She is undergoing chiropractic care. Confirms: nervousness and anxiety. Pt states that at this time she does not want a psych referral

Brain MRI impression: There are bifrontal subcortical foci of T2 hyperintensity seen within both cerebral hemispheres, particularly within the frontal lobes.

Plan
1. Continue all conservative measures.
2. Headache sheet reviewed. Recommend: 8hr of sleep a night; staying hydrated with water; avoiding highly processed foods, sugary foods and drinks, excessive caffeine, alcohol, nicotine. Magnesium Oxide 400mg-500mg PO at night with water to help aid in headache prevention. Taking walks as tolerated to help with increase oxygen intake as well as deep breathing exercises. Heat or ice on the neck area as needed every 2hrs for 30min to help aid in headache relief
3. The patient was advised to continue a MIND diet which is more important than ever during concussion recovery. The body and especially the brain need nutrients to recover from the injury. It is important to drink plenty of fluids to keep the body hydrated.
4. FU PRN if headaches return as pt states that she does not want to continue with care due to headaches resolving.

Continue chiro for ROM, stretching, strengthening, and endurance, continue low dose non-opioid pharmacotherapies as well as modalities including heat/ice and stretching, consider steroid TPI cervical paraspinals if symptoms persist/fail to respond

With a reasonable degree of medical certainty, there is a causal relationship between the accident and the patient's symptoms.

Treatment including home exercise program, chiropractic therapy, and non-opioid meds have been used. The patient is neurologically unchanged. Interventional procedures have been discussed with the patient.Good biomechanics were reviewed. Continue conservative therapies. The patient was instructed to call office if symptoms persist at this intensity or worsen. Risks and benefits of the medications were discussed and understanding was verbalized. The patient was advised to call and/or follow up earlier if any issues/concerns arose.

Plan is to do the multimodal approach to treatment which includes as needed procedures, home exercise physical therapy and as needed nonnarcotic low-dose medications for the short time to control and treat the pain.

Brain MRI reviewed with pt for which she verbalized understanding.

FU PRN if headaches return as pt states that she does not want to continue with care due to headaches resolving.

I spent 30 minutes today preparing the patient's chart, taking history, performing the exam, reviewing pertinent imaging with the patient, discussing diagnosis and recommend treatment options for management of symptoms, detailing recommended procedure/intervention with the use of spine/joint models for visual aid, and documenting in the EHR.

Cognitive Assessment

CPT Codes:
Moderate level established patient office visit (99213)

Follow up: -

Brittney Rish, NP
This has been electronically signed by Brittney Rish, NP on 10-22-2024.

3515 Braselton Hwy E2, Dacula GA 30019 - 5926

Tel: 7708004357, Fax:

INITIAL VISIT

Patient First Name:	**Patient Last Name:**	**Date of Birth:**	**Sex:**
Lewanda	Brown	08-26-1966	Female
Attending Provider:	**Referring Provider:**	**Visit Date:**	**Chart No.:**
Brittney Rish, NP		10-01-2024	SCL00371
Appointment Location:	**Appointment Location Address:**		
Neurospine Atlanta	3515 Braselton Hwy E2, Dacula GA 30019 - 5926		

Chief Complaint: Motor Vehicle Accident
Other Complaints: Headache

History of Present Illness
Motor Vehicle Accident
Lewanda Brown is a 58 year old female who was involved in a motor vehicle accident on 09-22-2024 in which she was the front seat passenger. At the time of accident patient was wearing a seatbelt. She suffered a rear impact to the vehicle. The accident occurred while the patient's vehicle was on the road. She denies airbag deployment. The patient was not braced for impact. She denies head trauma. She suffered no loss of consciousness. She was not evaluated in the ER. Went to urgent care the next day. Was given IBU and muscle relaxants.

Headache
Pt presents today for evaluation of headaches. Pt states that headaches started immediately after above described MVA. Pt states that she did not suffer any head trauma from MVA. Initially headaches were constant rated as a 10/10. As of today, headaches are improving but pt is still experiencing them 4 days a week rated as a 1-8/10 in intensity. Pt states that they vary in intensity and that IBU makes them tolerable. Headaches occur in the occipital and frontal regions and last 30min to all day. Her last headaches was last night. Pt is taking muscle relaxants at night to help with her pain. She is undergoing chiropractic care. She does not have photophobia. Patient does not have dizziness. There is no change in the patient's visual acuity. There is no vomiting associated with her headache. She does not report pain in her eyes. Patient denies having facial pain. She does not have ear pain. She is not having nasal stuffiness. Patient denies having nasal discharge. No lacrimation is reported.

Past Medical History
No Known Past Medical History

Surgical History

Lymph node removal may 2024

Family History

Non-contributory Family History.

Current Medication
IBU
methocarbamol

Allergy
No Known Drug Allergies.

Social History
Family: She is married.

Review of Systems
Constitutional Symptoms: Patients appears normal. Denies fever and fatigue.
HEENT: Denies headache, dizziness, blurry vision, light sensitivity, pain in eyes, Ear pain, Ear ringing and sensitivity to noise.
Respiratory: Denies trouble breathing.
Cardiovascular: Denies chest pain.
Neurological: Denies seizures, trouble with memory, trouble concentrating and poor coordination/imbalance.
Psychiatric: Reports **anxiety**, **nervousness** and **sleeping difficulty**. Denies Depressed mood and mood swings.

Vitals
Weight: 175.00 lbs. **Height:** 68.00 inches. **Pulse:** 70 per min. **BP Systolic:** 116 mmHg. **BP Diastolic:** 83 mmHg.

Physical Examination

General: The patient appears pleasant, in no apparent distress, well nourished and with good attention to hygiene and body habitus.
Neuro: Oriented to person, place and time.
Head: normocephalic, atraumatic
Respiratory: breathing comfortably on room air without evidence of labored breathing, cyanosis, and/or audible wheezing

CN:
II : Visual fields are intact on confrontation.
II-IV-VI: Extraocular movements are full without any nystagmus. Pupils are 3.5 mm in size, round, equal and reactive to light.
V: Facial sensations are intact bilaterally and symmetrically.
VII : Facial symmetry is intact bilaterally.
VIII: Hearing is intact bilaterally.
IX -X: Palate is symmetrical bilaterally.
XI: Shoulder shrug is equal and symmetrical bilaterally.
XII : Tongue is in midline.

Coordination :
Finger to nose, rapid alternating movements are intact.
Gait : Romberg is negative, patient is able to do tandem walking without any difficulties.
Motor system examination:

Tone is normal, strength is 5 over 5 in all the muscle groups in upper and lower extremities. DTRs : +2 equal and symmetrical in all muscle groups.
Plantars: These are flexor bilaterally.

Sensory system examination:
Touch, temperature, pinprick sensations are intact equally and symmetrically in upper and lower extremities; slightly diminished in LEFT upper extremity

minimal occipital nerve tenderness

Assessment
ICD: Post-traumatic headache (G44.309)
ICD: MVA (motor vehicle accident) (V89.2XXA)
ICD: Headache (R51.9)
ICD: Sleep difficulties (G47.9)
ICD: Anxiety (F41.9)
Pt presents today for evaluation of headaches. Pt states that she did not suffer any head trauma from MVA. Initially headaches were constant rated as a 10/10. As of today, headaches are improving but pt is still experiencing them 4 days a week rated as a 1-8/10 in intensity. Pt states that they vary in intensity and that IBU makes them tolerable. Headaches occur in the occipital and frontal regions and last 30min to all day. Her last headaches was last night. Pt is taking muscle relaxants at night to help with her pain. She is undergoing chiropractic care. Confirms: sleep difficulties, nervousness and anxiety.
Pt states that at this time she does not want a psych referral
Plan
1. Refer for brain MRI
2. Headache sheet given. Recommend: 8hr of sleep a night; staying hydrated with water; avoiding highly processed foods, sugary foods and drinks, excessive caffeine, alcohol, nicotine. Magnesium Oxide 400mg-500mg PO at night with water to help aid in headache prevention. Taking walks as tolerated to help with increase oxygen intake as well as deep breathing exercises. Heat or ice on the neck area as needed every 2hrs for 30min to help aid in headache relief
3. The patient was advised on a MIND diet which is more important than ever during concussion recovery. The body and especially the brain need nutrients to recover from the injury. It is important to drink plenty of fluids to keep the body hydrated.
4. Pt declined amitriptyline today as she would like to continue with more conservative measures.
5. FU 3 weeks for MRI review and further treatment options.

Continue chiro for ROM, stretching, strengthening, and endurance, continue low dose non-opioid pharmacotherapies as well as modalities including heat/ice and stretching, consider steroid TPI cervical paraspinals if symptoms persist/fail to respond

With a reasonable degree of medical certainty, there is a causal relationship between the accident and the patient's symptoms.

Treatment including home exercise program, chiropractic therapy, and non-opioid meds have been used. The patient is neurologically unchanged. Interventional procedures have been discussed with the patient.Good biomechanics were reviewed. Continue conservative therapies. The patient was instructed to call office if symptoms persist at this intensity or worsen. Risks and benefits of the medications were discussed and understanding was verbalized. The patient was advised to call and/or follow up earlier if any issues/concerns arose.

Plan is to do the multimodal approach to treatment which includes as needed procedures, home exercise physical therapy and as needed nonnarcotic low-dose medications for the short time to control and treat the pain.

FU in 3 weeks for MRI review and further treatment options.

I spent 45 minutes today preparing the patient's chart, taking history, performing the exam, reviewing pertinent imaging with the patient, discussing diagnosis and recommend treatment options for management of symptoms, detailing recommended procedure/intervention with the use of spine/joint models for visual aid, and documenting in the EHR.

CPT Codes:
Moderate level new patient office visit (99204)

Follow up: -

Brittney Rish, NP
This has been electronically signed by Brittney Rish, NP on 10-01-2024.

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

Initial Exam

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 9/28/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

She has never experienced this condition before.

PRIMARY COMPLAINT

Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT

Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT

Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: R Breast (2024)
Allergies: none
Medications: none
Prior conditions: none
Married, 3 children, Work: Cooking in a day care

Father: Alcoholism
Mother: Cancer

REVIEW OF SYSTEMS

MUSCULOSKELETAL: Muscle pain, Neck pain and Back pain

The following are negative: ALLERGIC-IMMUNOLOGIC, CARDIOVASCULAR, CONSTITUTIONAL, EAR/NOSE/THROAT, ENDOCRINE, EYES, GASTROINTESTINAL, GENITOURINARY, HEMATOLOGY/LYMPH, NEUROLOGICAL, PSYCHIATRIC, RESPIRATORY, SKIN, WOMEN'S HEALTH ISSUES and GENERAL.

Collision occurred on 9/22/2024 at approximately 10:12 AM.

Due to another accident ahead of us we were sitting still in traffic waiting on traffic to start moving and a car crashed in the back of us.
Symptoms reported: aches and pain in upper parts of body. (chest,back,and arms)

Ms. Brown was a passenger in the vehicle, a 2017 toyota thundra, at the time of the collision. There was rear, passenger side, bumper damage to the vehicle. The collision happened while she was stopped at Grayson Hwy in Lawrenceville , GA and occurred at night. The weather was clear. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were locked, and the car did not move. Ms. Brown was seated in the vehicle as a front passenger and was wearing a seat belt with the shoulder harness and with the headrest in down position. The vehicle is equipped with air bags and they did not deploy. Ms. Brown didn't see the impact coming. Upon impact, her head was looking ahead and she was thrown forward. Ms. Brown remembers the collision happening.
She remained in urgent car next day Northest georgia urgent care dacula ga for 1 hour
* Other instructions received: meds previouly prescribed should help with pain and discomfort.
Went to Urgent care in Dacula Ga. was treated there on 9/23/24 (morning after accident) for aches and pains and headache. was told to continue taking the ibuprofen 600, muscle relaxers and steroids that they had previously prescribed to me on the Thursday (9/19/24) prior to the accident due to sciatica nerve pain.
Performed Xray on the patient.. ROF to be performed on the next visit.The patient reports to this office due to injuries received in an MVC

OBJECTIVE

VITALS

Height: 65 in Weight: 175 lbs BMI: 29.1
Temperature: 96.8°F Pulse rate: 71 BPM (relaxed) SpO_2: 96%.
Blood Pressure (seated): right -N/A left-136/71 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Right Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Spasm/Hypertonicity

Left Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Right Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Trigger Points

Left Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

Right Side
- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back (mild-moderate)

GAIT

Ambulation: Normal.

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	Positive	
Foraminal compression test[2]	Positive	Positive	
O'Donogue maneuver[3]	Positive	Positive	
Shoulder depression test[4]	Positive	Positive	
Spinal percussion test-cervical[5]	Positive	Positive	
Schepelmann's sign[6]	Positive	Positive	
Spinal percussion test-thoracic[7]	Positive	Positive	
Kemp's test[8]	Positive	Positive	
Milgram's test[9]	Positive	Positive	
Spinal percussion test-lumbar[10]	Positive	Positive	
Straight leg raise[11]	Positive	Positive	
Bragard's test[12]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[4]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[5]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[6]Pain on the concave side may indicate intercostal neuritis. Pain on the convex side may be due to intercostal myofascitis.

[7]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[8]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[9]Lower back pain may indicate a herniated disc.

[10]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[11]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[12]Increased peripheral pain may indicate neuritis, spinal nerve irritation, or a space occupying lesion.

REFLEXES

Tricep and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	25° (50% of normal)	Pain, Spasm, Guarding	
Extension	35° (58% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	

Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	55° (68% of normal)	Pain, Spasm, Guarding	
Right rotation	55° (68% of normal)	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	35° (58% of normal)	Pain, Spasm, Guarding	
Extension	10° (40% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Right ateral flexion	10° (40% of normal)	Pain, Spasm, Guarding	
Left rotation	15° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	15° (50% of normal)	Pain, Spasm, Guarding	

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

Lower Muscle Strength

Location	Left	Right	Comment
Hip flexors	4/5	4/5	
Hip extensors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.
Cardiovascular – no carotid bruit or murmurs auscultated, regular rate and rhythm]: WNL.
Respiratory – [lungs clear to auscultation, no adventitious breath sounds noted, SPO2 >94%]: WNL.
Abdomen – [abdomen soft, nontender, bowl sounds present]: WNL.
There are no contraindications to chiropractic care.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)

5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings. The current complaints are a result of this MVC.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; C4; T3; T4; L1; L3; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Established Patient evaluation and management service, 99213, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 4 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/01/2024 09:16 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 9/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Performed Xray on the patient.. ROF to be performed on the next visit.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; **C3**; **C4**; T3; T4; L1; L4; L5; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Diagnostic Tests: X-ray cervical 2-3 views (72040) was provided due to cervical pain onset was immediate.**
- **Diagnostic Tests: X-ray thoracic 2 views (72070) was provided due to significant traumatic mechanism of injury.**
- **Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided due to significant traumatic mechanism of injury.**

Patient **completed X-rays today**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/01/2024 09:19 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Xrays were reviewed with the **patients**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; C3; **C5**; T3; T4; **T5**; L1; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Established Patient evaluation and management service, 99212, was provided today.**

Review x-rays with the patient. **The patient had an opportunity to ask questions and understand the findings**. **Due to positive findings on those** X-rays, **the patient will be referred for an MRI of the Cervical and Lumbar spine for further assessment**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/05/2024 11:41 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient was with **some left sciatic** pain.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C3; **C4**; **C6**; **C7**; T4; T5; **T6**; **T7**; **L2**; **L3**; L4; L5; **Right Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/09/2024 06:17 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Patient was with some stress and stiffness in the cervical region due to the need to purchase another vehicle due to the accident.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C3; C4; T5; T6; T7; L3; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/11/2024 10:20 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient stated that she was feeling discomfort on the left paraspinal musculature from the upper thoracic to the lower lumbar.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C3; C4; **Left T2**; **Left** T5; **Left** T6; **Left** T7; L3; **Left L4**; **Left L5**; **Sacrum**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/14/2024 10:31 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **is well with slight relief of her cervical and** lumbar **pain today**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to **C2**; **C3**; Left T2; Left T6; Left T7; **T8**; L3; Left L4; Left L5; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/16/2024 10:34 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/19/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **stated that she** is **feeling less discomfort and** with **more movement**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; **T1**; Left T2; Left T7; T8; L3; Left L4; Left L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/19/2024 10:55 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 7/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient stated that is feeling with **much relief this week**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; **C3**; Left T7; T8; Left L4; Left L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/22/2024 09:24 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 6 to 5/10 (10% decrease)**
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: **From 7 to 5/10 (20% decrease)**
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **only showed some signs of tightness however she** stated that **she had no soreness or discomfort at the moment**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; C3; **T6**; Left T7; T8; Left L4; Left L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/26/2024 09:48 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **was expressing** that **much relief and is happy how** she **has advanced in her reduction of pain**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; T6; Left T7; T8; Left L4; Left L5; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/30/2024 11:56 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 10/31/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient is **with laryngitis due to change** in **weather this week**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C2; **C3**; **T5**; T6; Left T7; Left L4; Left L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/31/2024 10:02 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Amanda Ruiz, DC
Date of Service: 11/1/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, **diversified technique**, **Thompson technique** was provided specifically to **C3**; **C4**; **T1**; **T2**; T5; T6; L4; **L5**; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/01/2024 05:38 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient has some cervical stiffness but besides that he has been feeling well.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T1; T2; **T3**; **T4**; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/04/2024 08:59 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **feels** that **her and her husband could** be **coming down with a cold**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be

performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.

- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C2**; C4; T1; T2; T3; **L3**; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/06/2024 08:51 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **was with some congestion toady**, **and feels** a **little muscle soreness**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; C3; **C4**; T2; T3; **T4**; **L2**; **L3**; **L4**; L5; Pelvis, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/14/2024 06:14 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Amanda Ruiz, DC
Date of Service: 11/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; C4; T1; T2; **Pelvis**; **Sacrum**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)

Provider Name: Amanda Ruiz, DC
Date: 11/09/2024 10:06 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/13/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient is doing well and seems to be holding her adjustments well.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T1; T2; **T3**; **L4**; Pelvis, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/14/2024 06:12 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient **mentioned no new changes since yesterdays** adjustment.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C2**; **C3**; T2; T3; **L5**; Pelvis, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/14/2024 06:13 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; **C4**; T3; **T4**; **T5**; **L3**; **L4**; **L5**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/15/2024 05:56 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient continues to be holding her adjustments well.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; C4; T4; T5; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/16/2024 12:32 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Lower Back (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
3. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
4. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
5. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
6. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
7. Segmental and somatic dysfunction of cervical region (M99.01)
8. Segmental and somatic dysfunction of thoracic region (M99.02)
9. Segmental and somatic dysfunction of lumbar region (M99.03)
10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; **C3**; **C4**; **T1**; **T2**; **T3**; T4; T5; **T10**; **T11**; **T12**; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/18/2024 05:36 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Mid back, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Low back

- Pain: 5/10
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

Patient continues holding her adjustments and feelin well.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
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10. Cervicalgia (M54.2)
11. Pain in thoracic spine (M54.6)
12. Low back pain, unspecified (M54.50)
13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T2; T3; T10; T11; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/20/2024 06:07 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Lewanda Brown
Date of Birth: 8/26/1966

Provider: Ricardo Heinis, DC
Date of Service: 11/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 5 to 1/10 (40% decrease)**
  Radiates to (posterior) left shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by **Chiropractic care** and rest.

SECOND COMPLAINT
Low back

- Pain: **From 5 to 1/10 (40% decrease)**
  Radiates to (posterior) left upper leg (thigh). Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by **Prescribed Pain Medication**, **Chiropractic care** and rest.

THIRD COMPLAINT
Mid back, Upper back

- Aching pain: **From 5 to 1/10 (40% decrease)**
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by **Chiropractic care** and rest.

Patient **was pleased with the outcome of** her **treatment**.

OBJECTIVE

VITALS
Height: 65 in Weight: 175 lbs BMI: 29.1
Temperature: 97.7°F Pulse rate: 94 BPM (relaxed) SpO_2: 99%.
Blood Pressure (seated): right -N/A left-163/91 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C2; C3; T1; T3; T4; T5; L4; L5**. No findings: C1; C4; C5; C6; C7; T2; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3**.

Tenderness

Left Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20% to minimal)**

Right Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20% to minimal)**

- Lower Back **(decreased 10-20% to minimal)**
- Lower Back **(decreased 10-20% to minimal)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20% to minimal)**
- Lower Back **(decreased 10-20% to minimal)**

Right Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20% to minimal)**
- Lower Back **(decreased 10-20% to minimal)**

Trigger Points

Left Side

- Cervical Spine **(decreased 20-35%)**
- Thoracic Spine **(decreased 20-35%)**
- Lower Back **(decreased 20-35%)**

Right Side

- Cervical Spine **(decreased 20-35%)**
- Thoracic Spine **(decreased 20-35%)**
- Lower Back **(decreased 20-35%)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Shoulder depression test[1]	Positive	**WNL**	
Kemp's test[2]	**WNL**	Positive	**mild soreness**
Straight leg raise[3]	**WNL**	Positive	**mild soreness**

[1]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[2]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[3]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

Cervical distraction , Foraminal compression test, O'Donogue maneuver, Spinal percussion test-cervical, Schepelmann's sign, Spinal percussion test-thoracic, Milgram's test, Spinal percussion test-lumbar and Bragard's test are within normal limits.

REFLEXES

Tricep and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Left lateral flexion	**40° (88% of normal, increased by 60%)**	Spasm, Guarding	
Left rotation	**75° (93% of normal, increased by 36%)**	Spasm, Guarding	
Right rotation	**75° (93% of normal, increased by 36%)**	Spasm, Guarding	

Flexion, **Extension** and **Right lateral flexion** are within normal limits.

Lumbar ROM - estimated

Flexion, **Extension**, **Left lateral flexion**, **Right ateral flexion**, **Left rotation** and **Right rotation** are within normal limits.

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and Cervical lateral flexon left/right are within normal limits bilaterally.

Lower Muscle Strength

Location	Left	Right	Comment
Hip flexors	4/5	4/5	
Hip extensors	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.
Cardiovascular – no carotid bruit or murmurs auscultated, regular rate and rhythm]: WNL.
Respiratory – [lungs clear to auscultation, no adventitious breath sounds noted, SPO2 >94%]: WNL.
Abdomen – [abdomen soft, nontender, bowl sounds present]: WNL.

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
2. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
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10. Cervicalgia (M54.2)
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12. Low back pain, unspecified (M54.50)

13. Muscle spasm of back (M62.830)
14. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PROGNOSIS

Prognosis has changed from **Guarded to Stable** - The patient has reached a plateau in their symptoms and may require continued care.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 90% of goal achieved.**

Patient is **showing improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T2; T3; T10; T11; L4; L5, to reduce pain reported at initial encounter by 50%.
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- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Final exam today. The patient has reached maximum medical improvement in this office. The patient will continue with home care instructions if symptoms persist.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/24/2024 09:29 PM

Addenda

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.

Brown, Lewanda — DOB: 8/26/1966 — Date of Service: 11/22/2024

Approximate $41,580 in conservative lifetime treatment costs.

Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X 12 X $150= Lifetime treatment.

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.

Electronic Signature: Heinis, Ricardo DC

Added on Nov 25, 2024